



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : حنان سعيد عمر ابراهيم
المحافظة : الجيزة
الحى / المدينة : بولاق الدكرور
العنوان : 0
رقم البطاقة : 26909040101523 رقم التليفون : 01156300093
تاريخ تسجيل الإستمارة : 26-09-2026
العمر (تاريخ الميلاد) : 04-09-1969 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: حنان سعيد عمر ابراهيم National ID: 26909040101523	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

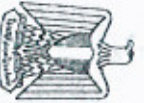
المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



 جمهورية مصر العربية
 بطاقة تحقيق الشخصية
 خان
 سعيد عمر ابراهيم
 هـ بن ابراهيم عطا
 بولاق النكرور - الجيزة
 ٢٦٩.٩.٤.٠١.٠١٥ ٢٣
 IX5663058



 ٢٠٢١/٠٦ ٢٦٩.٩.٤.٠١.٠١٥٢٣
 ربة منزل
 انش
 انور النور عمر ابراهيم
 البطاقة سارية حتى ٢٠٢٨/٠٥/٣١


 متروجة مسلمة


MDT FORM 23/12/2025 G

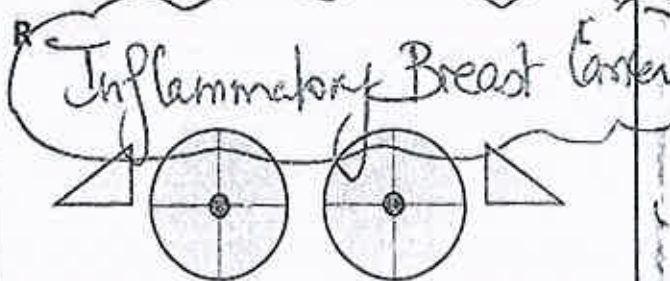
Name	مينا محمد عمر	MRN	3696/2025
Age	55y	ID	٤٦٩.٩.٤٠١.١٠٥٢
Menopausal status	Post hysterectomy	Phone	01156300095
NRS SCORE		Marital status	Married.
BMI		Family history	-ve
CO morbidity	HTN, DM	REASON OF MDT	New

Clinical Data

BX:	IDC G11
ER:	018
PR:	018
HER 2:	0
Ki67:	80+

Sonomammography:
④ 12 o'clock 3.5x2.5cm
2 o'clock 2.2x1.6cm

Mets Workup:
CT → Mediastinal Lymphadenopathy
fatty hepatomegaly



TNM
4d2

مدين جعفر

Recommendations:

MDT OUTCOME & PLAN

Peribronchovascular thickening
AC x4 cycles
→ then MRM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr. Mohamed Elmaghrabi	Dr. Ahmed Elmaghrabi				

ECHO REPORT

NAME:

حسان عبد عمر

DATE: 6/1/2026

ID NUMBER: ٢٦٩٦/٠٠٠

SEX: F AGE: 56

EF	65.1	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.6	3.5-5.8cm	AO.R	3.0	3-3.7cm
LVESD		2-4cm	LA	3.6	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions

LV Wall Thickness

LV Diastolic Function

SWMA

LA and Ao Root Dimensions

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

DR.

Julio

Patient Name : الأستاذة / حنان سعيد عمر
Patient No. : 26909040101523
Path No. : 3659/25
Sex : Female
Age : 56 Y
Referring Doc :

Request No. : 13003621
Request Date : 25/11/2025
Verification Date : 03/12/2025
Financial Category : جوسفات روسي - مبادره
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected on 25/11/2025, at 13:15 pm, Cold ischemia time: 30 seconds.

Gross Examination:

- Three grey white tissue cores, the longest is 1 cm. Totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for:

- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:

* Histological type: Invasive duct carcinoma.

* Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).

* DCIS: Minimal high grade ductal carcinoma insitu (Solid pattern).

* Necrosis: Minimal.

* Angioinvasion: Present.

* Perineural invasion: Absent.

* IPT (Positive control showed appropriate reactivity):

- ER: Negative nuclear staining in tumor cells (score 0/8).

- PR: Negative nuclear staining in tumor cells (score 0/8).

- Her2: Negative membrane staining in tumor cells (score 0).

- Ki67%: 80%.

Diagnosis:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma, Grade 3.

Verified by :

Dr.Duaa Mohamed

Printing Date : 06/12/2025 11:35:57

Page 1 of 3

Printing By : Mrs.Mayada Hegazy

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name : الأستاذ / حنان سعيد عمر
Patient No. : 26909040101523
Path No. : 3659/25
Sex : Female
Age : 56 Y
Referring Doc :

Request No. : 13003621
Request Date : 25/11/2025
Verification Date : 03/12/2025
Financial Category : جوسفات روسي - مبانره
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- Minimal high grade ductal carcinoma insitu (DCIS) (Solid pattern).
IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67% 80%

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

Printing Date : 06/12/2025 11:35:57

Page 2 of 3

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient ID: 90070	Patient Name English: Hanan Saled Omar
Age: ٥٩ سنة, ٧ شهر	2025/11/12 01:57:51 PM :Report Date

Women's Health Outreach Program
Bilateral Full field Digital Mammography

Clinical summary: 55 y old lady represented with left palpable lumps.

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Left breast

- Scattered dense fibro glandular parenchyma (ACR B).
- UOQ anterior third relatively high dense indistinct masses associated with skin thickening.
- **Ultrasound** demonstrated UOQ two irregular hypoechoic non parallel masses with posterior enhancement and internal vascularity noted at 12 o'clock (largest) measuring about 3.5x2.5cm and another one noted at 2 o'clock measuring about 2.2x1.6cm with 1cm distance inbetween.
- **Largest index mass being about 1.3cm from nipple and about 0.8cm from overlying skin.**
- **Mild skin thickening.**
- Level 1 enlarged pathological axillary LNs less than 3.

Right breast

- Scattered dense fibro glandular parenchyma (ACR B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Preserved axillary LNs.

Impression:

Left breast (BIRADS 5): UOQ multifocal malignant looking masses with ipsilateral pathological lymph nodes level 1.

Right breast (BIRADS 1): Normal study.

Recommendations:

Left UOQ biopsy and histopathological assessment.

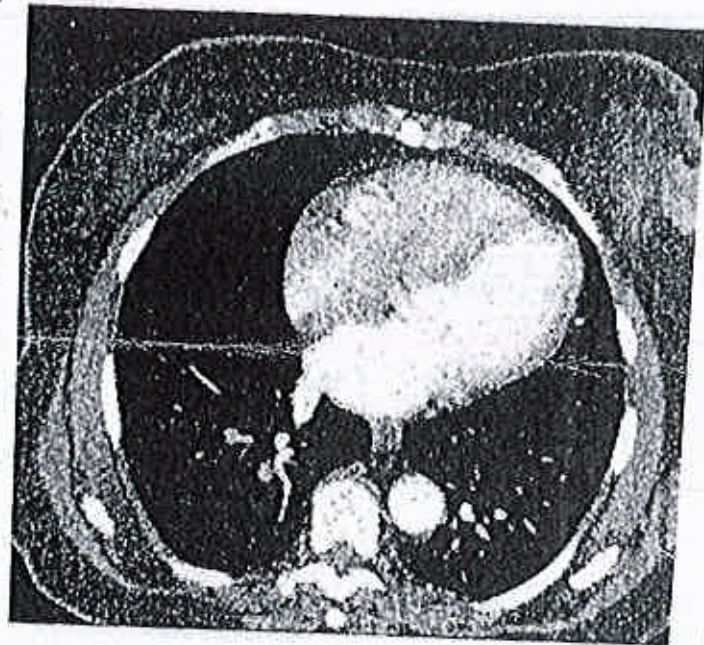
Dr.Reham Rady,FEBR,FFBR

Patient Name: Hanan Saeed Omar	Date : 20/12/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID: 3696/2025	Pat. ID: 133409

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Findings:

- Left breast shows retroareolar soft tissue mass measures 3.4x3.5 cm in axial cuts with skin thickening measures 15 mm.
- Left axillary and pectoral suspicious lymphadenopathy largest at left pectoral measures 17x15 mm and at left axillary measures 5.1x2.6 cm.
- Mediastinal lymphadenopathy largest is retrocaval measures 19x15 mm.
- Bilateral lower lobe mosaic attenuation and atelectatic bands.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Cardiomegaly for ECHO. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.



Patient Name: Hanan Saeed Omar	Date : 20/12/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID: 3696/2025	Pat. ID: 133409

- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Upper abdominal cuts are unremarkable.

OPINION:

Left breast shows retroareolar soft tissue mass for US correlation.
 Left axillary and pectoral suspicious lymphadenopathy
 Mediastinal lymphadenopathy
 Bilateral lower lobe mosaic attenuation for air trapping.
 Cardiomegaly for ECHO.

Dr. Mai Seddik

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER BOTH ORAL & I.V. CONTRAST ADMINISTRATION: -

Findings:

- The liver is enlarged in size with low homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.

Patient Name: Hanan Saeed Omar	Date : 20/12/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID: 3696/2025	Pat. ID: 133409

- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

Fatty hepatomegaly for lab correlation.

Dr Mai Mohamed MD

Dr. Mai Seddik

Signature Dr. Mohamed Hassan
22/12/2025 5:37 PM

Name : Hanan Said Omar Ibrahim

Date : 10-01-2026 10:36

Branch : Dokki

Medical No. : 291800

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعاقد مستشفيات

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- A 56-year-old lady, with history of left breast cancer.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions could be seen over the surveyed skeleton to account for metastatic bone disease.
- Marked degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from metastases.



Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

د. يوسف
روني
مستشار - جراحات الثدي



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

مستشفى
الجيزة
الجيزة

09/21/2026

D

MDT FORM

Name:	Hanan Saeed Omar *1\	MRN:	5918
Age:	57 Y	ID:	26909040101523
Menopausal Sta	postm	Phone:*	01156300093
NRS SCORE:		Marital Status:	
BMT:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:		postneoadj	
Surgical History:*	none	ECOG Performance Status	
		☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4	

Clinical Data:

R

L



T N M

BX:

ER: **PR:** **HER2:** **KI67:**

History:

postm rt breast cancer c4d n2 mo triple negative ki 80%
initial cts med ln 19x15
rec 12 weeks taxol carbopl ,4 ac last 8/2026 5 keytruda
last mdt for new cts then remdt
cts pretracheal lnodes 16x13 mm few subcentimetric pulmonary nodules
for remdt

Sonomammography:

Biomarker/Molecular

Met Workup:

Recommendations:

MDT Outcome & Plan

MRM AFTER EXAMINATION IN surgery clinic

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

Patient's Name:

Hanan Said Omar Ibrahim

Date:

2026-09-12

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

**MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA.

I- CHEST:

- Left breast heterogenous density with skin thickening for sono-mammography.
- Few tiny bilateral scattered pulmonary nodules measuring 2-3 mm in diameter.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- Pretracheal lymph node is seen measuring 16 x 13 mm.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing diffuse low parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Bilateral sacro-ileitis.

Opinion:

- ❖ Few tiny bilateral scattered pulmonary nodules likely inflammatory for follow up.
- ❖ Otherwise no pulmonary or hepatic metastatic lesions.

Pat. Name: Hanan Saeed Omar	Pat. ID: 3640292
Procedure Name: 3D Digital Mamo with Tomo -Bo	Date: 02/09/2026

3D FULL FIELD DIGITAL MAMMOGRAPHIC EXAMINATION OF BOTH BREASTS:

CLINICAL SUMMARY:

Left breast pathologically proven malignant mass under neoadjuvant chemotherapy.
For evaluation .
No previous study available for comparison.

IMPRESSION:

LEFT BREAST: (BIRADS 6)

- Middle retro areolar regions large hypoechoic area with UIQ and UOQ masses and prominent axillary lymph nodes cortices level I ,

RIGHT BREAST: (BIRADS 1)

- Normal study

RECOMMENDATION:-

- Comparison with previous unavailable study .
- Further contrast assessment

DETAILED FINDINGS:

TECHNIQUE:

Craniocaudal & mediolateral Oblique views
Tomosynthesis in 1mm slice thickness
High Resolution Ultrasound with Color Doppler

Both breasts showed dense fibro glandular pattern, (ACR c).

LEFT BREAST:

- Global increase density of the breast with diffuse skin thickening, obliterated subcutaneous fatty layer , trabecular and parenchyma thickening .

- This was confirmed by **3D Tomosynthesis 1mm section images.**
- **Complementary US** large branching hypoechoic areas occupying almost the middle third of the breast retro areolar region and forming irregular outlines faintly hypoechoic masses the largest at 11 O'clock measures 2.5x 2 cm and at 1-2 O'clock measures 2.5 x 1.2 cm .No hypervascularity by color doppler study partially extending to the nipple associated with skin thickening and consequent nipple retraction.
- Within average in size axillary lymph nodes the largest 1.2 cm with cortical thickness 3.8 mm .

RIGHT BREAST:

- No dominant masses, spiculations or suspicious microcalcifications.
- This was confirmed by **3D Tomosynthesis 1mm section images.**
- **Complementary US** demonstrated prominent fibro glandular parenchymal texture .
- No significant or suspicious masses detected.
- Normal skin/nipple/areolar complex.
- Preserved axillary lymph nodes.

Best regards,

Dr Suzan Mohamed

جوسلاف
روستني
مستشفى

9/15



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الجمهورية العربية السورية
مستشفى

09/02/2026

MDT FORM

Name:	Hanan Saeed Omar *1	MRN:	5918
Age:	57 Y	ID:	26909040101523
Menopausal Sta	POST	Phone:*	01156300093
NRS SCORE:		Marital Status:	MARRIED, 3 OFFSPRING
BMI:		Family History:	
Gender		Reason Of MDT:	
CO Morbidity:	DM HTN	POST NACT	
Surgical History:*	TAM	ECOG Performance Status	0 1 2 3 4

Clinical Data:

BX:

ER:

PR:

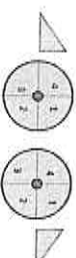
HER2:

KI67:

History:

55 yr old female, HTN, DM, newly dev hypothyroid (keytruda)
triple negative BC inflammatory breast cT4d N2 M0
received 12 wk TC 4 AC 6 KEYTRUDA last 2/8/2025,
Sonomammography:

T N M



Biomarker/Molecular

Q1 Q2 Q3 Q4 Right Side

Q1 Q2 Q3 Q4 Left Side

Mets Workup:

Recommendations:

for MRM

Clinical Staging:

MDT Outcome & Plan

initial CT reported mediastinal lymphadenopathy.
for new CT then re MDT

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
2	2		

Omar
219/2026